

CardioRAG-AGENT

A Tool-Using, Retrieval-Grounded, Self-Correcting Multimodal Reasoning Agent

Video Demo: <https://www.youtube.com/watch?v=0zYNCz7W3qw>

Code Base: <https://github.com/trankxu/uva-26-spring-gen-agent-students-projectB>

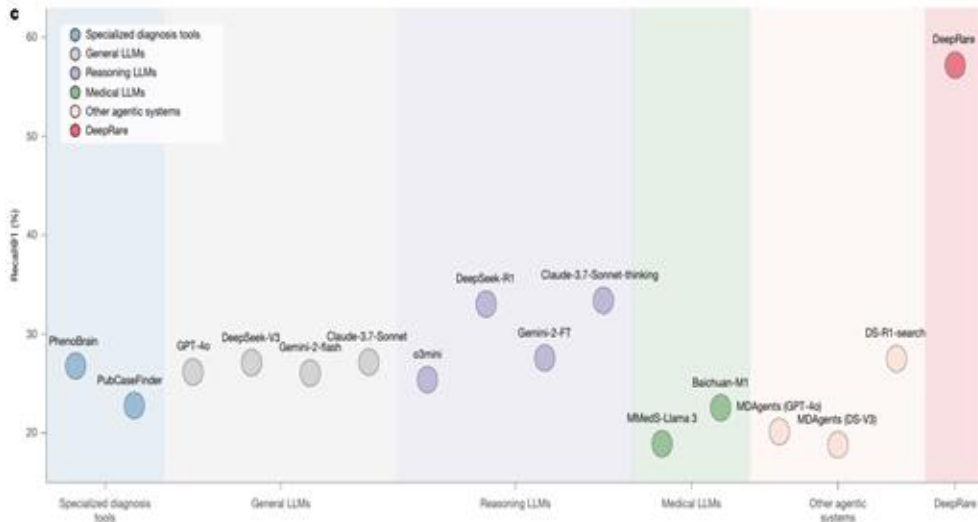
Team 6

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Motivation

● Why Agent?

- Limited specialist availability
- Rising clinical workload
- Diagnostic odyssey



● Current Medical Agent Gaps

The Modality Gap: Unimodal Ambiguity -> Multimodal Synergy

The Precision Gap: Weak Numerical Rigor -> Measurement Tool Use

The Grounded Gap: Ungrounded Hallucination -> Case-Based Retrieval (RAG)

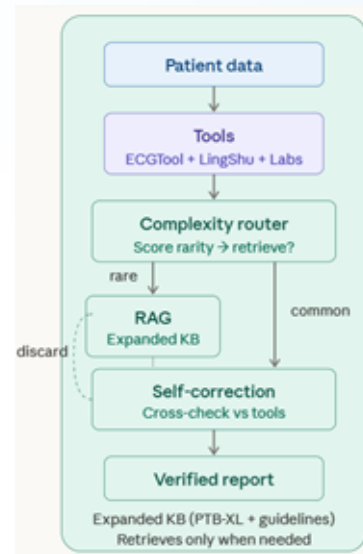
The Logic Gap: Physical Inconsistency -> Physics-Aware Self-Correction

Challenges in Rare Diseases: Naive RAG fetches misleading clinical mimics

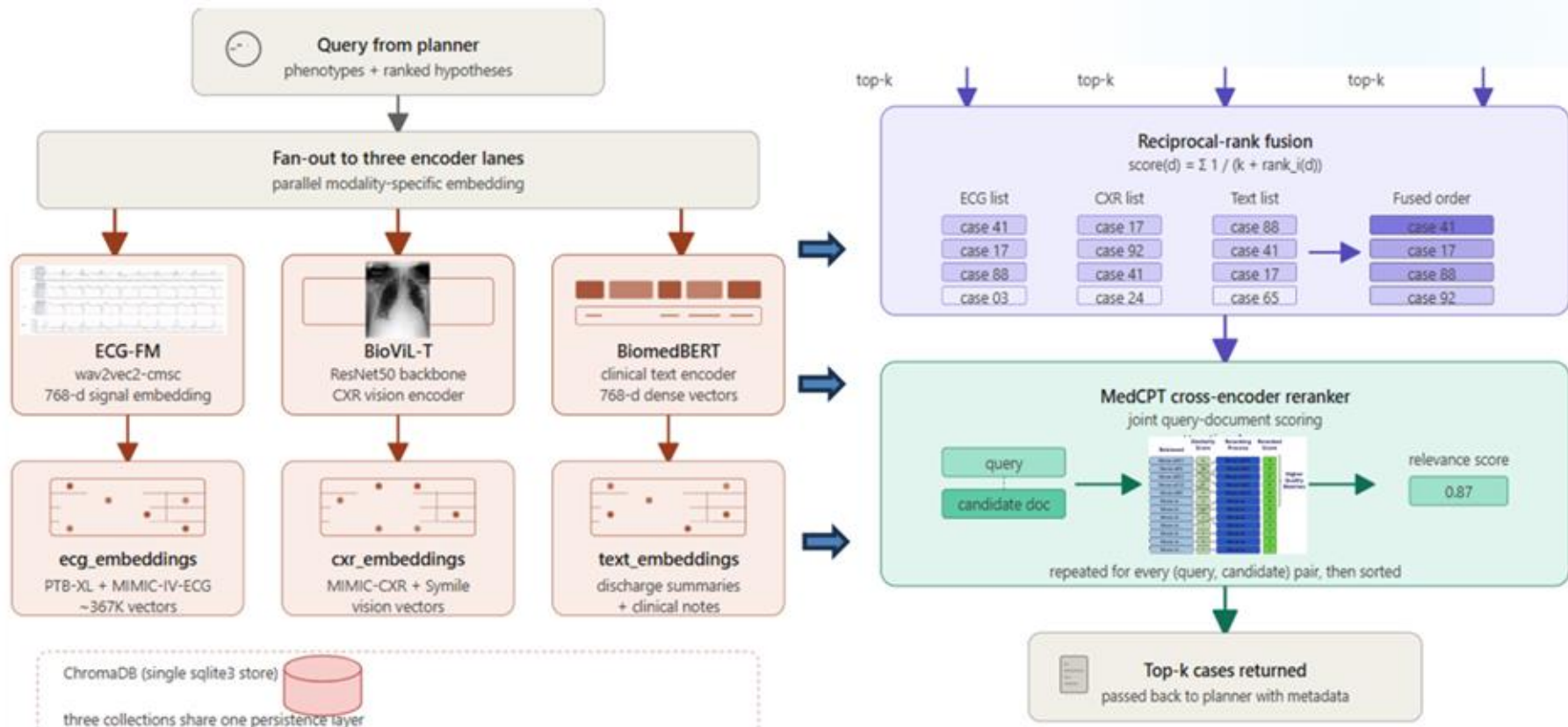
Problem Definition: What to Work On?

Expand and improve RAG utilization by enriching the knowledge base with general medical knowledge and larger historical case archives;

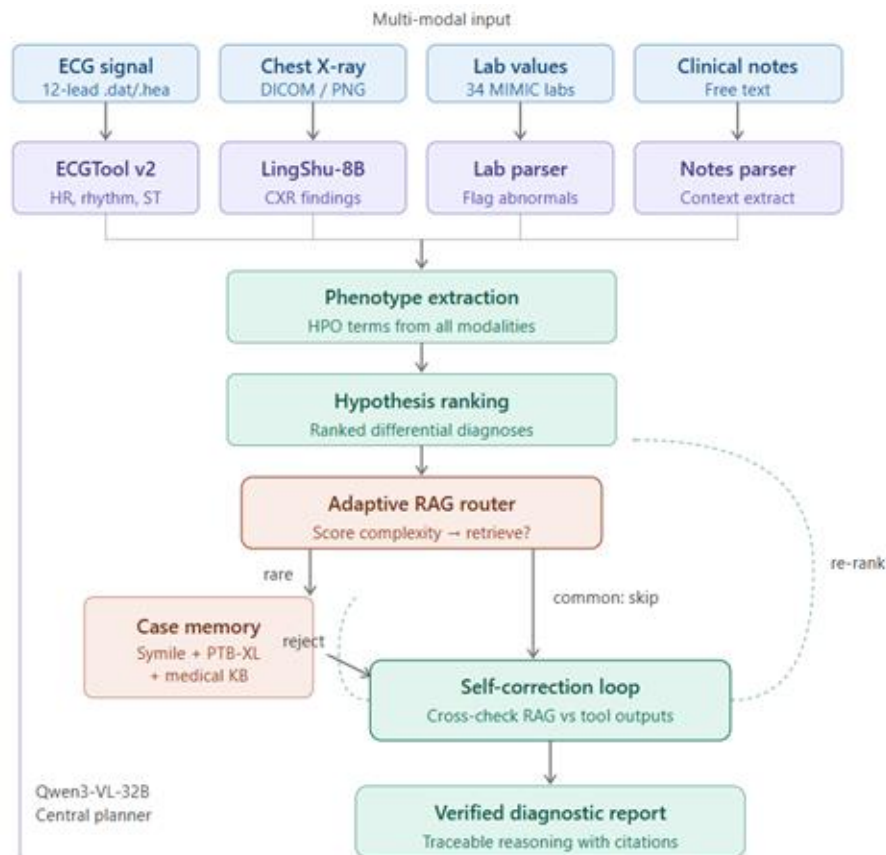
Adopt the *DeepRare*-inspired architecture to teach the Qwen Planner when to trigger retrieval, and how to critically evaluate retrieved cases against the patient's actual multi-modal findings before finalizing diagnosis.



System Architecture



System Architecture



Method - Structured Reasoning

Inspired by DeepRare, Nature 2025

1. Phenotype Extraction

Free text → HPO terms 38 rules

"Cardiomegaly"

→ HP:0001640 [cxr]

"BNP 892 pg/mL ↑"

→ HP:0430025 [lab]

"ST elevation V1-V3"

→ HP:0012251 [ecg]



2. Constellation Match

14 clinical rules pattern-match phenotype sets

Cardiomegaly + Edema + ↑BNP

→ CHF (0.85)

ST elevation + ↑Troponin

→ STEMI (0.85)

Tachycardia + ↑D-dimer

→ PE (0.50)



3. Traceable Hypothesis

Each diagnosis carries its evidence chain

DiagnosticHypothesis

diagnosis: CHF

confidence: 0.84

evidence: [

cxr → cardiomegaly

lab → BNP elevation

cross_modal: agree

]

Every prediction can be answered with: which modality saw what, and how strongly.

Method - Vector RAG

Multi-modal embedding retrieval — three specialized encoders fused via Reciprocal Rank Fusion

Knowledge Base: MIMIC-IV-ECG, MIMIC-IV-CXR, PTB-XL, Symile-MIMIC train set

Advantages: Keyword RAG misses synonyms — "heart failure" never matches "CHF", "MI", or "a-fib". Embeddings handle this natively.

ECG

ECG-FM

12-lead signal

Foundation model pre-trained on 1M+ ECGs.

IMAGE

BioViL-T

Chest X-ray

Biomedical vision-language transformer.

TEXT

BiomedBERT

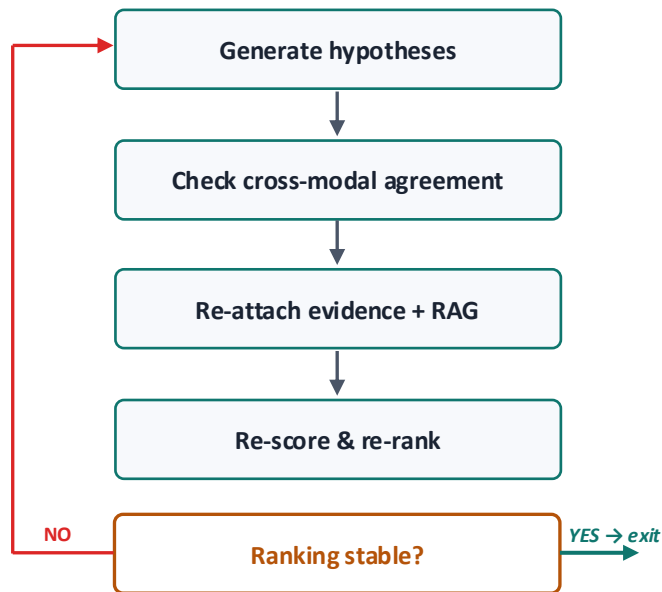
Notes + lab summary

Domain-specific text encoder.

RRF Fusion

$$\text{score}(c) = \sum \frac{1}{(k + \text{rank}_i(c))} \quad \text{sum across modalities} \rightarrow \text{top-k cases}$$

Method - Self-Reflection Loop



Re-score formula

$$\begin{aligned} \text{new} = & 0.50 \times \text{prior} \\ & + 0.15 \times \text{support} \\ & - 0.20 \times \text{contradict} \\ & + \min(0.10 \cdot n_{\text{modalities}}, 0.3) \\ & + \min(0.10 \cdot x_{\text{modal_agree}}, 0.2) \\ & + \min(0.05 \cdot \text{rag_match}, 0.15) \end{aligned}$$

Contradictions weighted higher than supports.

Source weights

ECG/Lab 1.0 CXR 0.9 Notes 0.7 RAG 0.4

Demo

End-to-end walkthrough on a Symile-MIMIC case

Patient ID

demo_patient_01

ECG (.hea+.dat,.npy,.edf)

47523342.dat
120.0KB

Chest X-Ray (.jpg,.png,.dcm)

7fc1b58...a3dc8d.jpg
1.1MB

Lab Results (.json,.csv,.txt)

blood_test.csv
2.3KB

Clinical Notes

e.g., 68yo M with acute chest pain,
SOB, diaphoresis...

Run Diagnostic Workflow

CONNECTING

CardioAgent

Multi-modal cardiac diagnostic agent DeepRare-inspired pipeline with transparent DST workflow and vector RAG retrieval

Running `load_planner(...)` - No module named 'chromadb'

Case Study: Diagnostic Trace (Sample 98)

Setting A Qwen-only

Cardiomegaly	0 ✓
Edema	0 ✗ missed
Atelectasis	0
Lung Opacity	1
No Finding	0
Pleural Effusion	0
35.7s Conservative — missed edema entirely	

Setting B +Tools, no RAG

Cardiomegaly	1 ✗ false positive
Edema	1 ✓
Atelectasis	1
Lung Opacity	1
No Finding	0
Pleural Effusion	1
84.9s Aggressive — caught edema but over-predicted	

Setting C +Tools +RAG

Cardiomegaly	1 ✗ false positive
Edema	1 ✓
Atelectasis	1
Lung Opacity	1
No Finding	0
Pleural Effusion	1
64.6s Same as B — RAG reinforced, didn't correct	

✓ Planning	4 modalities detected (ECG, Image, Labs, Notes)	
✓ ECG	HR 88 bpm, normal sinus rhythm, 1 finding	5.4s
✓ CXR (LingShu)	1 finding — but connection error noted in report	1.4s
✓ Labs	34 values, 7 abnormal : Cr↑ BUN↑ Lactate↑ WBC↑ Glucose↑ AST↑ ALP↑	0.0s
✓ Notes	267 chars — mentions cardiomegaly, edema, pleural effusion	0.0s
✓ Phenotypes	10 HPO terms: Cardiomegaly, Pulmonary edema, Pleural effusion, Atelectasis...	
✓ Hypotheses	CHF (0.75), Pneumonia (0.55), Cardiorenal syndrome (0.55)	
⚙ RAG	Skipped — CaseMemory not loaded	
✓ Reflection	2 rounds, 3 agreements, 0 contradictions → Cardiorenal syndrome (1.00)	
✓ Synthesis	5,352 chars — "Acute Decompensated Heart Failure with Cardiorenal Syndrome"	78.1s

Case 1: symile_train_24690376 → Lung Opacity

Case 2: symile_train_24494866 → Atelectasis, Cardiomegaly, Edema, Lung Opacity, Pleural Effusion

Case 3: symile_train_29481597 → Atelectasis, Cardiomegaly, Lung Opacity, Pleural Effusion

Case 4: symile_train_22072768 → Atelectasis, Cardiomegaly, Edema, Lung Opacity, Pleural Effusion

Case 5: symile_train_24217071 → Cardiomegaly, Edema, Lung Opacity, Pleural Effusion

4 of 5 retrieved cases have Cardiomegaly → RAG reinforced the false positive instead of correcting it.

Experiment Setup

Input Data Scope

Evaluation: MIMIC-IV-ECG (ECG) + Symile-MIMIC (CXR/Labs), test split only

External Knowledge: PTB-XL + MIMIC-IV-ECG + MIMIC-CXR-JPG, train split



ECG (Signal)

Electrical Function



CXR (Image)

Anatomical Structure



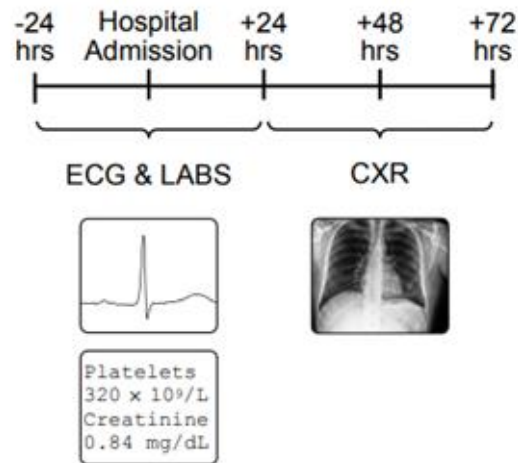
Doctor Notes (Text)

Ground Truth Context



Lab Results (Text)

Ground Truth Context



Evaluation - Ablation Study

Symile-MIMIC test set; MIMIC-IV-ECG test data

Component	A	B	C	D
Qwen3-VL zero-shot	✓	✓	✓	✓
ECGTool + Lingshu imaging	—	✓	✓	✓
Phenotypes + Reflection	—	✓	✓	✓
Keyword RAG	—	—	✓	—
Vector RAG (RRF)	—	—	—	✓
Research question	Baseline	Agentic?	Keyword?	Vector?

Evaluation - Ablation Study

Symile-MIMIC test set; MIMIC-IV-ECG test data

Setting	Description	n	Macro AUROC	Macro AUPRC	Macro F1	Micro AUROC	Exact Match	Hamming ↓
<i>Block 1 — Pipeline ablation (200 samples)</i>								
A	Qwen3-VL zero-shot — CXR + labs only, no tools, no RAG	187 / 200	0.4709	0.6441	0.5438	0.5307	0.2437	0.5340
B	DeepRare planner + ECG/CXR tools — phenotype extraction + reflection, no RAG	200 / 200	0.5443	0.6808	0.7144	0.5257	0.3163	0.4339
C	DeepRare + naïve text RAG (367K) — keyword retrieval over MIMIC-IV-ECG	200 / 200	0.5588	0.6883	0.7221	0.5304	0.3214	0.4355
<i>Block 2 — Vector RAG modality ablation (2,000 samples)</i>								
D1	ECG-FM only — single-modality ECG embeddings	2000 / 2000	0.5891	0.7101	0.7579	0.5518	0.3720	0.4058
D2	BioViL-T (CXR) only — single-modality CXR embeddings	2000 / 2000	0.5867	0.7082	0.7563	0.5450	0.3654	0.4077
D3	BiomedBERT (text) only — single-modality clinical text embeddings	2000 / 2000	0.5761	0.7058	0.7527	0.5451	0.3674	0.4095
D4	All modalities + RRF fusion — reciprocal-rank fusion across ECG / CXR / text	2000 / 2000	0.5829	0.7073	0.7534	0.5463	0.3679	0.4111

Takeaway & Future Work

- Medical Agent means traceable reasoning, not just better prompt.
- Full models (with vector RAG) work the best.
- For these CXR labels, ECG already carries most of the signal, so adding text and image rankings just adds noise

Future Work

- Refine the front-end interface
- Improve performance
- Add more rules

Thank you! Any Questions ?